SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Adjuvant abemaciclib + endocrine therapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HR-positive / HER2-negative early breast cancer · adjuvant · NCCP 00619 · version 4a · 28-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
For eligible high-risk node-positive early breast cancer, adding abemaciclib to endocrine therapy reduces the risk of recurrence after surgery.

**Schedule**
Abemaciclib is taken twice daily continuously, with endocrine treatment, for up to two years if appropriate. A 28-day block is shown for orientation; there is no planned tablet-free week. Endocrine treatment usually continues longer.

## 1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

### Abemaciclib
**CDK4/6 INHIBITOR**

- ☐ Diarrhoea, dehydration and nausea
- ☐ Low blood counts and infection
- ☐ Liver-test abnormalities
- ☐ Blood clots
- ☐ Rare serious lung inflammation

### Your endocrine treatment
**HORMONE-DIRECTED TREATMENT**

- ☐ Hot flushes, joint or muscle symptoms
- ☐ Vaginal dryness and sexual effects
- ☐ Bone loss with AI / ovarian suppression
- ☐ Clot and uterine risks with tamoxifen

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions, the planned monitoring and who to contact. Review the full longitudinal pathway and confirm which conditional treatments apply.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00619 v4a.

SACTCheck v0.83.7
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